

India's Trusted Health Test @Home Service



NABL
ACCREDITED



Booking ID : 17618795239

Sample Collection Date : 04/Apr/2026

Miss SNEHA VERMA

Female, 21 Yrs 11 Days

A Comprehensive Health Analysis Report

AI Based Personalized Report for You

Flagship Lab in Gurugram

CAP
ACCREDITED
COLLEGE of AMERICAN PATHOLOGISTS
CAP #: 9019582 | AU-ID: 2107746



INDIA'S FIRST & ONLY CREDIBILITY CHECK FOR YOUR LAB REPORT

Check the authenticity of your lab report with machine data

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MACHINE & QC DETAILS

LABORATORY EQUIPMENT DASHBOARD

Miss SNEHA VERMA

Booking ID : 17618795239



Beckman AU5800

Beckman Coulter : Beckman AU5800

Department - Biochemistry

No. of test(s) performed - 31

Performed Test(s): **Liver Function Test, Kidney Function Test (KFT), Lipid Profile**

Description: The Beckman Coulter clinical chemistry analyzer manufactured in USA, is the fastest in the industry, offering a throughput of 2,000 tests per hour using spectrophotometry and 2900 tests per hour with ISE. It utilizes advanced spectrophotometric technology to ensure high accuracy and reliability, delivering over 21 parameters that meet Six Sigma performance standards.

Frequency of Quality Control (QC) : **3 times a day** (Industry best and as per NABL guidelines)



Biorad D100

Bio-Rad : Biorad D100

Department - BIOCHEMISTRY HBA1C

No. of test(s) performed - 2

Performed Test(s): **HbA1c**

Description: The Bio-Rad fully automated HPLC-based HbA1c analyzer manufactured in USA is a gold-standard system for accurate HbA1c testing. It is NGSP certified, IFCC traceable, and unaffected by common haemoglobin variants, ensuring highly reliable results.

Frequency of Quality Control (QC) : **Twice a day** (Industry best and as per NABL guidelines)



Horiba Yumi1500

HORIBA : Horiba Yumi1500

Department - Hematology

No. of test(s) performed - 24

Performed Test(s): **COMPLETE BLOOD COUNT**

Description: The fully automated high-end hematology analyzer manufactured in France, features a patented Double Hydrodynamic Sequential System (DHSS) for precise, linear cell flow and enhanced accuracy. It determines total WBC counts using both impedance and flow cytometry technologies. The system provides detailed immature cell analysis—including immature granulocytes, monocytes, lymphocytes, and large immature cells—for improved sepsis screening and cancer pre-screening. Its 360° automated, homogeneous sample mixing minimizes manual intervention and preanalytical errors.

Frequency of Quality Control (QC) : **Twice a day** (Industry best and as per NABL guidelines)



Alifax

Suyog Diagnostics : Alifax

Department - Hematology

No. of test(s) performed - 1

Performed Test(s): **ESR Automated**

Description: The Alifax Test 1 is a next-generation, fully automated ESR analyzer manufactured in Italy. Utilizing quantitative capillary photometry, it measures red blood cell aggregation kinetics, addressing many limitations of the traditional Westergren method, offering fast, accurate, and reliable ESR testing for modern clinical laboratories.

Frequency of Quality Control (QC) : **Twice a day** (Industry best and as per NABL guidelines)



Beckman AU700

Beckman Coulter : Beckman AU700

Department - Biochemistry

No. of test(s) performed - 1

Performed Test(s): **Blood Glucose Fasting**

Description: The Beckman Coulter state-of-the-art clinical chemistry analyzer manufactured in USA, delivers exceptional performance with a throughput of up to 800 tests per hour using spectrophotometry and 1,200 tests per hour with ISE, ensuring outstanding diagnostic accuracy and precision. Utilizing advanced spectrophotometric technology, it provides reliable and highly accurate results, supporting over 21 parameters that meet Six Sigma performance standards for superior laboratory efficiency and quality.

Frequency of Quality Control (QC) : **3 times a day** (Industry best and as per NABL guidelines)

HEALTH ANALYSIS

Personalized Summary & Vital Parameters

Miss SNEHA VERMA

Booking ID : 17618795239 | Sample Collection Date : 04/Apr/2026

Miss SNEHA VERMA,

Congratulations, We have successfully completed your health diagnosis. This is a big step towards staying on top of your health and identify potential to improve!

10 Vital Health Parameters of a Human Body Ecosystem

Below are the health parameters which require routine checkups for primary healthcare. The view also includes *personalised information* depending on the tests you have taken.

Your Health Score

89
Out of 100

*Calculated from test reports



Thyroid Function

Thyroid Stimulating Hormone (TSH) - Ultrasensitive : 2.91 μ IU/ml

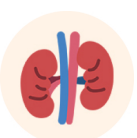
● Everything looks good



Cholesterol Total

116 mg/dl

● Everything looks good



Kidney Function

Serum Creatinine : 0.37 mg/dl

● Everything looks good



Vitamin D

7.13 ng/ml

● Concern



HbA1c

4.5 %

● Everything looks good



Vitamin B12

Test not taken



Liver Function

Alanine Aminotransferase (ALT/SGPT) : 8.6 U/L

● Everything looks good



Calcium Total

9 mg/dl

● Everything looks good



Iron studies

Test not taken



Complete Hemogram

Haemoglobin (HB) : 13.1 g/dL

● Everything looks good





New Features

Report Summary

Miss SNEHA VERMA

Booking ID 17618795239 | Sample Collection Date: 04/Apr/2026

Understanding laboratory reports can be complex, often leading to unwarranted anxiety.

At Healthians, we understand that you shouldn't have to rely on a Google search to decipher your own health report. That's why we offer comprehensive summaries that are easy to understand.

Health Test Report Summary for Miss Sneha Verma

Dear Miss Verma,

I have reviewed your recent health test results. It's completely normal to have a few values that fall outside the typical range, and I want to reassure you that these findings are often manageable and do not necessarily indicate serious health issues. We'll focus on understanding these results and how to best support your well-being.

Summary of Deranged Parameters

Your results show a few areas that are a little different from the usual. Your Vitamin D level is lower than what is typically considered optimal, which is quite common. Your HDL cholesterol, often referred to as "good" cholesterol, is also a bit lower than the standard range, and the ratio of your total cholesterol to HDL is also on the lower side. Additionally, your serum chloride level is slightly elevated, and your mean platelet volume is a bit higher than usual. You also have a slightly higher percentage of lymphocytes, which are a type of white blood cell, and the ratio of your SGOT to SGPT enzymes is also a little elevated.

These findings are being brought to your attention so we can work together to optimize your health.

Suggestions for Your Well-being

Here are a few suggestions to help address these findings and support your overall health:

- 1. Focus on Vitamin D Rich Foods and Sunlight Exposure:** To help improve your Vitamin D levels, consider incorporating more Vitamin D-rich foods into your diet, such as fatty fish, fortified dairy products, and eggs. Sensible sun exposure, when possible, can also be beneficial.
- 2. Emphasize Heart-Healthy Lifestyle Choices:** To support your cholesterol levels, focus on a diet rich in fruits, vegetables, whole grains, and lean proteins. Regular physical activity is also very important for maintaining healthy cholesterol.
- 3. Stay Well-Hydrated and Maintain a Balanced Diet:** Ensuring you are drinking an adequate amount of fluids throughout the day can help with electrolyte balance. A balanced diet with a variety of nutrients is key to supporting your body's functions.
- 4. Consider Stress Management Techniques:** Given the slight elevations in your MPV and lymphocyte percentage, and the SGOT/SGPT ratio, incorporating stress-reducing activities into your routine can be beneficial. Practices like mindfulness, deep breathing exercises, or gentle yoga can help promote overall balance.

Patient Name : Miss SNEHA VERMA
Age/Gender : 21Y 0M 11D /Female
Order Id : 17618795239
Referred By : Self
Customer Since : 04/Apr/2026
Sample Type : EDTA Plasma

Barcode : E6610590
Sample Collected On : 04/Apr/2026 09:49AM
Sample Received On : 04/Apr/2026 03:34PM
Report Generated On : 04/Apr/2026 06:52PM
Sample Temperature : Maintained ✓
Report Status : Final Report

DEPARTMENT OF BIOCHEMISTRY HBA1C

Test Name	Value	Unit	Bio. Ref Interval
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HbA1c - Glycosylated Hemoglobin

HbA1c (Glycosylated Hemoglobin)	4.50	%	4.2 - 5.7
Method: HPLC			
Machine: BIORAD D100			
Average Estimated Glucose - plasma	82.45	mg/dl	
Method: Calculated			

INTERPRETATION:

AS PER AMERICAN DIABETES ASSOCIATION (ADA):

REFERENCE GROUP

Non diabetic
At Risk (Prediabetes)
Diagnosing Diabetes

GLYCOSYLATED HEMOGLOBIN (HBA1c) in %

<5.7

5.7 – 6.4

>= 6.5

Age > 19 Years

Goals of Therapy: < 7.0

Actions Suggested: >8.0

Age < 19 Years

Goal of therapy: <7.5

Therapeutic goals for glycemic control

REMARKS

- HbA1c is used for monitoring diabetic control. It reflects the mean plasma glucose over three months
- HbA1c may be falsely low in diabetics with hemolytic disease. In these individuals a plasma fructosamine level may be used which evaluates diabetes over 15 days.
- Inappropriately low HbA1c values may be reported due to hemolysis, recent blood transfusion, acute blood loss, hypertriglyceridemia, chronic liver disease. Drugs like dapson, ribavirin, antiretroviral drugs, trimethoprim, may also cause interference with estimation of HbA1c, causing falsely low values.
- HbA1c may be increased in patients with polycythemia or post-splenectomy.
- Inappropriately higher values of HbA1c may be caused due to iron deficiency, vitamin B12 deficiency, alcohol intake, uremia, hyperbilirubinemia and large doses of aspirin.
- Trends in HbA1c are a better indicator of diabetic control than a solitary test.
- Any sample with >15% HbA1c should be suspected of having a hemoglobin variant, especially in a non-diabetic patient. Similarly, below 4% should prompt additional studies to determine the possible presence of variant hemoglobin.
- HbA1c target in pregnancy is to attain level <6 % .
- HbA1c target in paediatric age group is to attain level < 7.5 %.
- Method : Ion-exchange high-performance liquid chromatography (HPLC).
- Reference : American Diabetes Associations. Standards of Medical Care in Diabetes 2023
- The presence of some hemoglobinopathies/hemoglobin variants may be missed by some platforms used for measuring HbA1c values. In these cases, the HbA1c result may vary or give deviant results, depending on the platform used. Alternative measures of glycemic control like fructosamine or glycated albumin may be considered in such cases.
- In presence of hemoglobinopathy/hemoglobin variants in homozygous/compound heterozygous conditions alternative forms of testing such as fructosamine or glycated albumin may be considered.

Gautam

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MBBS, MD Pathology
Consultant Pathologist
Reg. No.-17509, Healthians Labs



SIN No:E6610590

Healthians Labs (A Unit of Expedient Healthcare Marketing Pvt. Ltd.)

Plot 1 & 2, Udyog Vihar, Phase-4, Gurgaon, Haryana, Pincode-122016 (CAP Number: 9019582 | NABL Accreditation Certificate Number MC-4245)

Patient Name : Miss SNEHA VERMA
Age/Gender : 21Y 0M 11D /Female
Order Id : 17618795239
Referred By : Self
Customer Since : 04/Apr/2026
Sample Type : Sodium Fluoride

Barcode : E6610590
Sample Collected On : 04/Apr/2026 09:49AM
Sample Received On : 04/Apr/2026 03:33PM
Report Generated On : 04/Apr/2026 04:12PM
Sample Temperature : Maintained ✓
Report Status : Final Report

DEPARTMENT OF BIOCHEMISTRY

Test Name	Value	Unit	Bio. Ref Interval
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Fasting Blood Sugar

Glucose, Fasting	78.8	mg/dl	70 - 100
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Method: Hexokinase
Machine: BECKMAN COULTER DXC 700 AU

American Diabetes Association Reference Range :

Normal : < 100 mg/dl
Impaired fasting glucose(Prediabetes) : 100 - 125 mg/dl
Diabetes : >= 126 mg/dl

Conditions that can result in an elevated blood glucose level include:

- Diabetes mellitus ,Hemochromatosis ,Cushing syndrome ,Acromegaly and gigantism.
- Increased circulating epinephrine such as in pheochromocytoma and adrenalin injections
- Acute pancreatitis
- Chronic pancreatitis

Conditions that cause low blood glucose level include :

- Pancreatic disorders : Islet cell tumor , pancreatitis
- Hepatic disease (diffuse severe disease)
- Endocrine disorders : hypopituitarism, Addison's disease ,hypothyroidism
- Alcoholism
- Malnutrition

Preeti

Dr. Preeti
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Consultant Biochemist
DMC - 97534, Healthians Labs



SIN No:E6610590



Patient Name : Miss SNEHA VERMA
Age/Gender : 21Y 0M 11D /Female
Order Id : 17618795239
Referred By : Self
Customer Since : 04/Apr/2026
Sample Type : Serum

Barcode : E6610590
Sample Collected On : 04/Apr/2026 09:49AM
Sample Received On : 04/Apr/2026 03:44PM
Report Generated On : 04/Apr/2026 06:50PM
Sample Temperature : Maintained ✓
Report Status : Final Report

DEPARTMENT OF BIOCHEMISTRY

Test Name	Value	Unit	Bio. Ref Interval
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Liver Function Test (LFT)

Serum Bilirubin, (Total) Method: DPD Machine: BECKMAN COULTER AU 5800	0.66	mg/dl	0.3 - 1.2
Serum Bilirubin, (Direct) Method: DPD Machine: BECKMAN COULTER AU 5800	0.15	mg/dl	0 - 0.2
Serum Bilirubin, (Indirect) Method: Calculated	0.51	mg/dl	0.0 - 0.8
Aspartate Aminotransferase (AST/SGOT) Method: UV without P5P Machine: BECKMAN COULTER AU 5800	15.50	U/L	3 - 35
Alanine Aminotransferase (ALT/SGPT) Method: UV without P5P Machine: BECKMAN COULTER AU 5800	8.6	U/L	3 - 35
Alkaline Phosphatase (ALP) Method: AMP Buffer Machine: BECKMAN COULTER AU 5800	52.00	U/L	33-98
Gamma Glutamyl Transferase (GGT) Method: IFCC Machine: BECKMAN COULTER AU 5800	10.4	U/L	5- 38
Serum Total Protein Method: BIURET Machine: BECKMAN COULTER AU 5800	6.93	gm/dl	6.6 - 8.3
Serum Albumin Method: BROMOCRESOL GREEN Machine: BECKMAN COULTER AU 5800	3.83	g/dl	3.5 - 5.2
Serum Globulin Method: Calculated	3.10	gm/dl	3.0 - 4.2
Albumin/Globulin Ratio Method: Calculated	1.24	Ratio	1.2 - 2.5
SGOT/SGPT Ratio Method: Calculated	1.80	Ratio	0.7 - 1.4

Bilirubin is a yellowish pigment found in bile and is a breakdown product of normal heme catabolism. Elevated levels are a result of increased bilirubin production (e.g hemolysis and ineffective erythropoiesis), decreased bilirubin excretion (e.g.; obstruction and hepatitis) and abnormal bilirubin metabolism (e.g; hereditary and neonatal jaundice) .

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SIN No:E6610590



Patient Name	: Miss SNEHA VERMA	Barcode	: E6610590	
Age/Gender	: 21Y 0M 11D /Female	Sample Collected On	: 04/Apr/2026 09:49AM	
Order Id	: 17618795239	Sample Received On	: 04/Apr/2026 03:44PM	
Referred By	: Self	Report Generated On	: 04/Apr/2026 06:50PM	
Customer Since	: 04/Apr/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF BIOCHEMISTRY

Test Name	Value	Unit	Bio. Ref Interval
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Conjugated (direct) bilirubin is elevated in conditions like- Hereditary disorders(Dubin Johnson syndrome, Rotor syndrome),Hepatocellular damage(e.g –viral ,toxic ,alcohol ,drugs) ,biliary duct obstruction (extrahepatic or intrahepatic), Infiltrations ,space occupying lesions(e.g metastasis, abscess , granuloma , amyloidosis. Increased unconjugated (indirect) bilirubin may be a result of hemolytic or pernicious anemia, transfusion reaction & a common metabolic condition termed Gilbert syndrome.

AST levels increase in viral hepatitis, blockage of the bile duct ,cirrhosis of the liver, liver cancer, kidney failure, hemolytic anemia, pancreatitis, hemochromatosis. AST levels may also increase after a heart attack or strenuous activity.

ALT is a liver specific enzyme commonly measured as a part of a diagnostic evaluation of hepatocellular injury, to determine liver health.

Elevated ALP levels are seen in Biliary Obstruction, Osteoblastic Bone Tumors, Osteomalacia, Hepatitis, Hyperparathyroidism, Leukemia, Lymphoma, Paget's disease, Rickets, Sarcoidosis etc.

Elevated serum GGT activity can be found in diseases of the liver, Biliary system and pancreas. Obstructive liver disease, high alcohol consumption and use of enzyme-inducing drugs lead to raised GGT levels .

Serum total protein measures the total amount of protein in serum. It is largely comprised of albumin and globulins. Increased levels may be due to: Chronic inflammation or infection, including HIV and hepatitis B or C, multiple myeloma, Waldenstrom's disease. Decreased levels may be due to: Agammaglobulinemia, Bleeding (hemorrhage), Burns, Glomerulonephritis, Liver disease, Malabsorption, Malnutrition, Nephrotic Syndrome.

Albumin is the most abundant protein in the serum and is produced in the liver. Low serum albumin levels (hypoalbuminemia) can be caused by: Liver diseases like liver cirrhosis, nephrotic syndrome, protein-losing enteropathy, burns, hemodilution, increased vascular permeability or decreased lymphatic clearance, malnutrition and wasting .

Globulins are increased in most liver diseases , in chronic inflammatory diseases and neoplastic diseases.

Please note that there is a change in the Biological Reference Interval for Alkaline Phosphatase (ALP) parameter. (Dated: 11.02.2026).



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SIN No:E6610590

Patient Name : Miss SNEHA VERMA
Age/Gender : 21Y 0M 11D /Female
Order Id : 17618795239
Referred By : Self
Customer Since : 04/Apr/2026
Sample Type : Serum

Barcode : E6610590
Sample Collected On : 04/Apr/2026 09:49AM
Sample Received On : 04/Apr/2026 03:44PM
Report Generated On : 04/Apr/2026 06:55PM
Sample Temperature : Maintained ✓
Report Status : Final Report

DEPARTMENT OF BIOCHEMISTRY

Test Name	Value	Unit	Bio. Ref Interval
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(KFT) Kidney Function Test

Serum Creatinine Method: Modified Jaffe, Kinetic Machine: BECKMAN COULTER AU 5800	0.37	mg/dl	0.3 - 1.2
GFR, ESTIMATED Method: Calculated (CKD-EPI)	147.03	mL/min/1.73m2	
Serum Uric Acid Method: Uricase PAP Machine: BECKMAN COULTER AU 5800	4.7	mg/dl	2.6-6.0
Serum Calcium Method: ARSENazo III Machine: BECKMAN COULTER AU 5800	9.0	mg/dl	8.8 - 10.6
Serum Sodium Method: ISE (Indirect) Machine: BECKMAN COULTER AU 5800	137	mmol/L	136 - 146
Serum Chloride Method: ISE (Indirect) Machine: BECKMAN COULTER AU 5800	111	mmol/L	101 - 109
Blood Urea Method: GLDH, Kinetic assay Machine: BECKMAN COULTER AU 5800	17.4	mg/dl	17 - 43
Blood Urea Nitrogen (BUN) Method: Calculated	8.1	mg/dl	8 - 20
Bun/Creatinine Ratio Method: Calculated	21.97	Ratio	
Urea/Creatinine Ratio Method: Calculated	47.03	Ratio	

The kidneys play a vital role in the excretion of waste products and toxins such as urea, creatinine and uric acid, regulation of extracellular fluid volume, serum osmolality and electrolyte concentrations, as well as the production of hormones like erythropoietin and 1,25 dihydroxy vitamin D and renin. Assessment of renal function is important in the management of patients with kidney disease or pathologies affecting renal function. Tests of renal function have utility in identifying the presence of renal disease, monitoring the response of kidneys to treatment, and determining the progression of renal disease.

Urea is synthesized in the liver as the final product of protein and amino acid metabolism. Urea synthesis is therefore dependent on daily protein intake and endogenous protein metabolism.

Creatinine is a metabolic product of creatine and phosphocreatine, which are both found almost exclusively in muscle.

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SIN No:E6610590



Patient Name : Miss SNEHA VERMA
Age/Gender : 21Y 0M 11D /Female
Order Id : 17618795239
Referred By : Self
Customer Since : 04/Apr/2026
Sample Type : Serum

Barcode : E6610590
Sample Collected On : 04/Apr/2026 09:49AM
Sample Received On : 04/Apr/2026 03:44PM
Report Generated On : 04/Apr/2026 06:55PM
Sample Temperature : Maintained ✓
Report Status : Final Report

DEPARTMENT OF BIOCHEMISTRY

Test Name	Value	Unit	Bio. Ref Interval
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Uric Acid is the major product of purine catabolism in humans. Uric acid levels are used to monitor the treatment of gout.

Measurement of calcium is used in the diagnosis and treatment of parathyroid disease, a variety of bone diseases, chronic renal disease, urolithiasis and tetany. Phosphorus levels are increased in acute or chronic renal failure with decreased GFR .

Sodium is an electrolyte, and it helps regulate the amount of water in and around the cells & the balance of chemicals in the body called acids and bases. Potassium is a primary intracellular ion, only 2 % is extracellular, high intracellular concentration is maintained by a Na- K ATPase pump, which continuously transports potassium into the cell against a concentration gradient. Chloride is a negatively charged ion that works with other electrolytes such as potassium, sodium, and bicarbonate, to help regulate the amount of fluid in the body and maintain the acid-base balance.

EGFR-National kidney Disease Education program recommends the use of MDRD equation to predict/ estimate GFR in adults (>=20 years) with Chronic kidney disease. Calculated by serum creatinine may be less accurate due to certain factor like race muscle mass, diet and certain drugs in such case EGFR should be calculated using serum cystatin C.

Age (Years)	Male ML/min/1.73m ²	Female ML/min/1.73m ²
17-24	93-131	—
25-34	78-146	—
35-44	74-138	65-123
45-54	74-129	58-110
55-64	69-122	50-110
65-74	61-114	46-105
75-84	52-102	48-85

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SIN No:E6610590



Patient Name : Miss SNEHA VERMA
Age/Gender : 21Y 0M 11D /Female
Order Id : 17618795239
Referred By : Self
Customer Since : 04/Apr/2026
Sample Type : Serum

Barcode : E6610590
Sample Collected On : 04/Apr/2026 09:49AM
Sample Received On : 04/Apr/2026 03:44PM
Report Generated On : 04/Apr/2026 06:50PM
Sample Temperature : Maintained ✓
Report Status : Final Report

DEPARTMENT OF BIOCHEMISTRY

Test Name	Value	Unit	Bio. Ref Interval
Lipid Profile			
Total Cholesterol Method: CHO-POD (Trac. Abel-Kendall) Machine: BECKMAN COULTER AU 5800	116.0	mg/dl	Desirable : <200 Borderline: 200-239 High : >=240
Serum Triglycerides Method: GPO-POD Machine: BECKMAN COULTER AU 5800	97.9	mg/dL	Desirable : <150 Borderline high : 150-199 High : 200-499 Very high : >= 500
Serum HDL Cholesterol Method: ENZYMATIC IMMUNOINHIBITION Machine: BECKMAN COULTER AU 5800	39.3	mg/dL	40 - 60
LDL Cholesterol Calculated Method: Calculated	57.12	mg/dL	Optimal : <100 near /above Optimal:100 - 129 Borderline High: 130- 159 High : 160 - 189 Very High :>=190
VLDL Cholesterol Calculated Method: Calculated	19.58	mg/dl	<30
Total CHOL / HDL Cholesterol Ratio Method: Calculated	2.95	Ratio	3.30 - 4.40
LDL / HDL Cholesterol Ratio Method: Calculated	1.45	Ratio	Desirable/Low Risk: 0.5-3.0 Line/Moderate Risk: 3.0-6.0 Elevated/High Risk: >6.0
HDL / LDL Cholesterol Ratio Method: Calculated	0.69	Ratio	Optimal->0.4 Moderate-0.4 to 0.3 High-<0.3
Non-HDL Cholesterol Method: Calculated	76.7	mg/dL	0.0 - 160.0

Dyslipidemia is a disorder of fat or lipoprotein metabolism in the body and includes lipoprotein overproduction or deficiency. Dyslipidemias means increase in the level of one or more of the following: Total Cholesterol, low density lipoprotein (LDL) and/or triglyceride concentrations. Dyslipidemia also includes a decrease in the "good" cholesterol or high-density lipoprotein (HDL) concentration in the blood. Cholesterol is a steroid carried in the bloodstream as lipoprotein, necessary for cell membrane functioning and as a precursor to bile acids, progesterone, vitamin D, estrogens, glucocorticoids and mineralocorticoids. HDL is termed "good cholesterol" because its levels are inversely related to the risk of Coronary heart disease.

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Patient Name : Miss SNEHA VERMA
Age/Gender : 21Y 0M 11D /Female
Order Id : 17618795239
Referred By : Self
Customer Since : 04/Apr/2026
Sample Type : Serum

Barcode : E6610590
Sample Collected On : 04/Apr/2026 09:49AM
Sample Received On : 04/Apr/2026 03:44PM
Report Generated On : 04/Apr/2026 06:50PM
Sample Temperature : Maintained ✓
Report Status : Final Report

DEPARTMENT OF BIOCHEMISTRY

Test Name	Value	Unit	Bio. Ref Interval
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LDL cholesterol is termed the "bad cholesterol" and their increased levels are associated with increased risk of atherosclerosis and coronary heart disease. Lipid level assessments must be made following 9 to 12 hours of fasting, otherwise assay results might lead to erroneous interpretation. Healthians labs report biological reference intervals (normal ranges) in accordance with the recommendations of The National Cholesterol Education Program (NCEP) & Adult Treatment Panel IV (ATP IV) guidelines providing the most desirable targets of various circulating lipid fractions in the blood. NCEP recommends that all adults above 20 years of age must be screened for abnormal lipid levels.

Updated risk stratification approach recommended by the Lipid Association of India (2023)

Risk Category	
Extreme Risk	Category A: ASCVD with ≥ 1 feature of high-risk group •CACS ≥300 •Homozygous FH
	Category B: ASCVD with ≥ 1 feature of very high-risk group or • Recurrent ACS or • Polyvascular disease or •Homozygous FH
	Category C: Recurrent ASCVD event despite LDL-C around 30 mg/dL. These patients require special consideration.
Very High Risk (VHR)	•Diabetes with target organ damage • Diabetes with ≥2 major ASCVD risk factors •CACS 100–299 or >75 percentile if CACS 1–99 •≥2 high risk features •Established ASCVD (obstructive or non-obstructive) •Heterozygous FH or LDL-C ≥190 mg/dL
High Risk (HR)	•>3 major ASCVD risk factors, or •LDL-C 160–189 mg/dL or •Non-HDL-C 190–219 mg/dL or •Diabetes with 0–1 major ASCVD risk factor •2 major ASCVD factors + ≥1 risk modifier or •Any 1 high-risk feature.
Moderate Risk (MR)	•2 major ASCVD risk factors, or •LDL-C 130–159 mg/dL or •Non-HDL-C 160–189 mg/dL or •Low-risk group + ≥1 risk modifier or lifetime ASCVD risk >30%
Low Risk (LR)	•0–1 major ASCVD risk factor and •LDL-C 100–129 mg/dL and •Non-HDL-C 130–159 mg/dL, and •Lifetime CVD risk <30%

Major Risk ASCVD (Atherosclerotic Cardiovascular Disease) Risk Factors

•Age ≥45 years in males and ≥55 years in females •Current cigarette smoking or tobacco use •High blood pressure •Low HDL-C

Treatment targets for lipid lowering therapy for various atherosclerotic cardiovascular disease risk groups

Risk Groups	Treatment Targets		
	LDL-C, mg/dl (Primary Target)	Non-HDL-C,mg/dl(Coprimary Target)	Apo-B, mg/dl (Secondary Target)
Low risk group	<100	<130	<90
Moderate risk group	<100 (optional <70)	<130 (optional <100)	<90
High risk group	<70	<100	<80
Very high risk	<50	<80	<65
Extreme -risk-group- Category (A)	<50 (optional ≤30)	<80 (optional ≤60)	<65
Extreme -risk-group- Category (B)	≤30	≤60	<50
Extreme -risk-group- Category (C)	10-15	40-45	-----

Reference: Journal of Clinical Lipidology, Vol-18 No-3. May/June 2024

Preeti
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DMC - 97534, Healthians Labs



SIN No:E6610590



Patient Name	: Miss SNEHA VERMA	Barcode	: E6610590	
Age/Gender	: 21Y 0M 11D /Female	Sample Collected On	: 04/Apr/2026 09:49AM	
Order Id	: 17618795239	Sample Received On	: 04/Apr/2026 03:28PM	
Referred By	: Self	Report Generated On	: 04/Apr/2026 07:46PM	
Customer Since	: 04/Apr/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Urine	Report Status	: Final Report	

DEPARTMENT OF CLINICAL PATHOLOGY

Test Name	Value	Unit	Bio. Ref Interval
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Urine Routine & Microscopy Extended

PHYSICAL EXAMINATION

Colour	Pale Yellow		Pale Yellow
Method: Visual			
Volume	15.00	mL	
Method: Visual			
Appearance	Slightly Hazy		Clear
Method: Visual			

CHEMICAL EXAMINATION

Specific Gravity	1.020		1.001 - 1.035
Method: Dipstick-Ion exchange			
Machine: URIPLUS_600			
pH	5.5		4.5 - 7.5
Method: Dipstick-Double indicator			
Machine: URIPLUS_600			
Glucose	Negative		Negative
Method: Dipstick-oxidase peroxidase			
Machine: URIPLUS_600			
Urine Protein	Negative		Negative
Method: Dipstick-Bromophenol blue			
Machine: URIPLUS_600			
Ketones	Negative		Negative
Method: Sodium nitroprusside			
Machine: URIPLUS_600			
Urobilinogen	Normal		Normal
Method: Dipstick-Ehrlichs Test			
Machine: URIPLUS_600			
Bilirubin	Negative		Negative
Method: Dipstick-Ehrlichs Test			
Machine: URIPLUS_600			
Nitrite	Negative		Negative
Method: Dipstick-Griess test			
Machine: URIPLUS_600			
Blood	Trace		Nil
Method: Dipstick-Peroxidase			

Gautam

Dr. Harsh Vardhan Gautam
MBBS, MD Pathology
Consultant Pathologist
Reg. No.-17509, Healthians Labs



SIN No: E6610590

Patient Name : Miss SNEHA VERMA
Age/Gender : 21Y 0M 11D /Female
Order Id : 17618795239
Referred By : Self
Customer Since : 04/Apr/2026
Sample Type : Urine

Barcode : E6610590
Sample Collected On : 04/Apr/2026 09:49AM
Sample Received On : 04/Apr/2026 03:28PM
Report Generated On : 04/Apr/2026 07:46PM
Sample Temperature : Maintained ✓
Report Status : Final Report

DEPARTMENT OF CLINICAL PATHOLOGY

Test Name	Value	Unit	Bio. Ref Interval
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Machine: URIPLUS_600

Leucocyte Esterase

Negative

Nil

Method: Dipstick- Esterase

Machine: URIPLUS_600

MICROSCOPIC EXAMINATION

Pus Cells

4-5

/HPF

0 - 5

Method: Microscopic Examination

Epithelial cells

2-3

/HPF

0 - 5

Method: Microscopic Examination

RBCs

6-8

/HPF

Nil

Method: Microscopic Examination

Casts

Nil

Nil

Method: Microscopic Examination

Crystals

Nil

Nil

Method: Microscopic Examination

Bacteria

Absent

Absent

Method: Microscopic Examination

Yeast Cell

Absent

Absent

Method: Microscopic Examination

Others (Non Specific)

Nil

NIL

Method: Microscopic Examination

The main indication for testing for glucose in urine is detection of unsuspected diabetes mellitus or follow-up of known diabetic patients. Renal glycosuria accounts for 5% of cases of glycosuria in general population.

Proteinuria can be seen in nephrotic syndrome, pyelonephritis, heavy metal poisoning, tuberculosis of kidney, interstitial nephritis, cystinosis, Fanconi syndrome, rejection of kidney transplant. Hemodynamic proteinuria is transient and can be seen in high fever, hypertension, heavy exercise, congestive cardiac failure, seizures, and exposure to cold. Post-renal proteinuria is caused by inflammatory or neoplastic conditions in renal pelvis, ureter, bladder, prostate, or urethra.

Ketonuria can be seen in uncontrolled Diabetes mellitus with ketoacidosis, Glycogen storage disorder, starvation, persistent vomiting in children, weight reduction program, fever in children, severe thyrotoxicosis, pregnancy and protein calorie malnutrition.

Presence of bilirubin in urine indicates conjugated hyperbilirubinemia (obstructive or hepatocellular jaundice). Bile salts along with bilirubin can be detected in urine in cases of obstructive jaundice. Normally about 0.5-4 mg of urobilinogen is excreted in urine in 24 hours. Therefore, a small amount of urobilinogen is normally detectable in urine. Increased urobilinogen in urine can be seen due to hemolysis, megaloblastic anemia and haemorrhage in tissues. Decreased urobilinogen can be seen in obstructive jaundice, reduction of intestinal bacterial flora, neonates and following antibiotic treatment. The presence of abnormal number of intact red blood cells in urine is called as hematuria. It implies presence of a bleeding lesion in the urinary tract. Hematuria can be seen in glomerular diseases like Glomerulonephritis, Berger's disease, lupus nephritis, Henoch-Schonlein purpura, non glomerular diseases like Calculus, tumor, infection, tuberculosis, pyelonephritis, hydronephrosis, polycystic kidney disease, trauma, after strenuous physical exercise, diseases of prostate (benign hyperplasia of prostate, carcinoma of prostate).

Nitrites are not present in normal urine. Ingested nitrites are converted to nitrate and excreted

in urine. If gram-negative bacteria (e.g. E.coli, Salmonella, Proteus, Klebsiella, etc.) are present in urine, they will reduce the nitrates to nitrites through the action of bacterial enzyme nitrate reductase. As E. coli is the commonest organism causing urinary tract infection, this test is helpful as a screening test for urinary tract infection.

Some organisms like Staphylococci or Pseudomonas do not reduce nitrate to nitrite and therefore in such infections nitrite test is negative.

Leucocyte esterase test detects esterase enzyme released in urine from granules of leucocytes. Thus the test is positive in pyuria.

Dr. Harsh Vardhan Gautam

Dr. Harsh Vardhan Gautam
MBBS, MD Pathology
Consultant Pathologist
Reg. No.-17509, Healthians Labs



SIN No:E6610590

Patient Name	: Miss SNEHA VERMA	Barcode	: E6610590	
Age/Gender	: 21Y 0M 11D /Female	Sample Collected On	: 04/Apr/2026 09:49AM	
Order Id	: 17618795239	Sample Received On	: 04/Apr/2026 03:28PM	
Referred By	: Self	Report Generated On	: 04/Apr/2026 07:46PM	
Customer Since	: 04/Apr/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Urine	Report Status	: Final Report	

DEPARTMENT OF CLINICAL PATHOLOGY

Test Name	Value	Unit	Bio. Ref Interval
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Gautam

Dr. Harsh Vardhan Gautam
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Reg. No.-17509, Healthians Labs



SIN No:E6610590

Patient Name : Miss SNEHA VERMA
Age/Gender : 21Y 0M 11D /Female
Order Id : 17618795239
Referred By : Self
Customer Since : 04/Apr/2026
Sample Type : Whole Blood EDTA

Barcode : E6610590
Sample Collected On : 04/Apr/2026 09:49AM
Sample Received On : 04/Apr/2026 03:34PM
Report Generated On : 04/Apr/2026 05:41PM
Sample Temperature : Maintained ✓
Report Status : Final Report

DEPARTMENT OF HAEMATOLOGY

Test Name	Value	Unit	Bio. Ref Interval
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ERYTHROCYTE SEDIMENTATION RATE (ESR)

ESR	03	mm/1st hour	0-12
Method: Kinetic Red Cells Aggregation			
Machine: ALIFAX TEST - 1			

Gautam

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SIN No: E6610590



Patient Name : Miss SNEHA VERMA
Age/Gender : 21Y 0M 11D /Female
Order Id : 17618795239
Referred By : Self
Customer Since : 04/Apr/2026
Sample Type : Whole Blood EDTA

Barcode : E6610590
Sample Collected On : 04/Apr/2026 09:49AM
Sample Received On : 04/Apr/2026 03:34PM
Report Generated On : 04/Apr/2026 04:38PM
Sample Temperature : Maintained ✓
Report Status : Final Report

DEPARTMENT OF HAEMATOLOGY

Test Name	Value	Unit	Bio. Ref Interval
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Complete Blood Count

Haemoglobin (HB) Method: Spectrophotometry Machine: Yumizen H1500	13.1	g/dL	12.0-15.0
Total Leucocyte Count (TLC) Method: Impedance Machine: Yumizen H1500	5.4	10 ³ /uL	4.0-10.0
Hematocrit (PCV) Method: Calculated Machine: Yumizen H1500	39.7	%	36.0-46.0
Red Blood Cell Count (RBC) Method: Impedance Machine: Yumizen H1500	4.25	10 ⁶ /μl	3.80-4.80
Mean Corp Volume (MCV) Method: Derived from RBC Histogram Machine: Yumizen H1500	93.4	fL	83.0-101.0
Mean Corp Hb (MCH) Method: Calculated Machine: Yumizen H1500	30.9	pg	27.0-32.0
Mean Corp Hb Conc (MCHC) Method: Calculated Machine: Yumizen H1500	33.0	g/dL	31.5-34.5
RDW - CV Method: Derived from RBC Histogram Machine: Yumizen H1500	12.7	%	11.6-14.0
RDW - SD Method: Derived from RBC Histogram Machine: Yumizen H1500	40.80	fL	39.0-46.0
Mentzer Index Method: Calculated	21.98	Ratio	
RDWI Method: Calculated	279.10	Ratio	
Green and king index Method: Calculated	85	Ratio	

Differential Leucocyte Count

Neutrophils	47.8	%	40 - 80
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Gautam

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SIN No: E6610590

Patient Name : Miss SNEHA VERMA
Age/Gender : 21Y 0M 11D /Female
Order Id : 17618795239
Referred By : Self
Customer Since : 04/Apr/2026
Sample Type : Whole Blood EDTA

Barcode : E6610590
Sample Collected On : 04/Apr/2026 09:49AM
Sample Received On : 04/Apr/2026 03:34PM
Report Generated On : 04/Apr/2026 04:38PM
Sample Temperature : Maintained ✓
Report Status : Final Report

DEPARTMENT OF HAEMATOLOGY

Test Name	Value	Unit	Bio. Ref Interval
Method: Flow-Cytometry DHSS Machine: Yumizen H1500			
Lymphocytes	43.7	%	20-40
Method: Flow-Cytometry DHSS Machine: Yumizen H1500			
Monocytes	7.1	%	02 - 10
Method: Flow-Cytometry DHSS Machine: Yumizen H1500			
Eosinophils	1.0	%	01 - 06
Method: Flow-Cytometry DHSS Machine: Yumizen H1500			
Basophils	0.4	%	00 - 02
Method: Impedance Machine: Yumizen H1500			
Absolute Leucocyte Count			
Absolute Neutrophil Count (ANC)	2.59	10 ³ /uL	2.0-7.0
Method: Calculated			
Absolute Lymphocyte Count (ALC)	2.36	10 ³ /uL	1.0-3.0
Method: Calculated			
Absolute Monocyte Count	0.38	10 ³ /uL	0.2-1.0
Method: Calculated			
Absolute Eosinophil Count (AEC)	0.05	10 ³ /uL	0.02-0.5
Method: Calculated			
Absolute Basophil Count	0.02	10 ³ /uL	0.02 - 0.10
Method: Calculated			
Platelet Count(PLT)	263	10 ³ /μl	150-410
Method: Impedance Machine: Yumizen H1500			
MPV	9.7	fL	7 - 9
Method: Derived from PLT Histogram Machine: Yumizen H1500			

The International Council for Standardization in Haematology (ICSH) recommends reporting of absolute counts of various WBC subsets for clinical decision making. This test has been performed on a fully automated 5 part differential cell counter which counts over 10,000 WBCs to derive differential counts. A complete blood count is a blood panel that gives information about the cells in a patient's blood, such as the cell count for each cell type and the concentrations of Hemoglobin and platelets. The cells that circulate in the bloodstream are generally divided into three types: white blood cells (leukocytes), red blood cells (erythrocytes), and platelets (thrombocytes). Abnormally high or low counts may be physiological or may indicate disease conditions, and hence need to be interpreted clinically.

The Mentzer index is used to differentiate iron deficiency anaemia beta thalassemia trait. If a CBC indicates microcytic anaemia, these are two of the most likely

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SIN No: E6610590

Patient Name	: Miss SNEHA VERMA	Barcode	: E6610590	
Age/Gender	: 21Y 0M 11D /Female	Sample Collected On	: 04/Apr/2026 09:49AM	
Order Id	: 17618795239	Sample Received On	: 04/Apr/2026 03:34PM	
Referred By	: Self	Report Generated On	: 04/Apr/2026 04:38PM	
Customer Since	: 04/Apr/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Whole Blood EDTA	Report Status	: Final Report	

DEPARTMENT OF HAEMATOLOGY

Test Name	Value	Unit	Bio. Ref Interval
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causes, making It necessary to distinguish between them.

If the quotient of the mean corpuscular volume divided by the red blood cell count is then 13, thalassemia is more likely. If the result is greater than 13, then iron deficiency anaemia is more likely.

Green and King Index used to differentiate IDA from thalassemia trait value >65 is likely to be Iron Deficiency Anemia and value <220 more likely to be Beta Thalassemia Trait.

Calculated parameters are either derived from Impedence measure, RBC pulse measurement, RBC/platelet histograms or formula derived.

Gautam

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SIN No: E6610590

Patient Name	: Miss SNEHA VERMA	Barcode	: E6610590	
Age/Gender	: 21Y 0M 11D /Female	Sample Collected On	: 04/Apr/2026 09:49AM	
Order Id	: 17618795239	Sample Received On	: 04/Apr/2026 03:44PM	
Referred By	: Self	Report Generated On	: 04/Apr/2026 06:42PM	
Customer Since	: 04/Apr/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF IMMUNOLOGY

Test Name	Value	Unit	Bio. Ref Interval
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Thyroid Stimulating Hormone (TSH)

Thyroid Stimulating Hormone (TSH)-Ultrasensitive	2.909	μIU/ml	0.38-5.33
Method: CLIA			
Machine: Beckman Coulter UniCel DxI800			

Interpretation:

Pregnancy interval	Bio Ref Range for TSH in uIU/ml (As per American Thyroid Association)
First trimester	0.1 - 2.5
Second trimester	0.2 – 3.0
Third trimester	0.3 – 3.0

1. Thyroid hormones undergo circadian variation: Peak levels are seen at around midnight. Minimum levels seen between morning through noon. This variation may be as much as 50% thus, influence of sampling time needs to be considered for clinical interpretation.
2. Circulating forms of T3 and T4 are mostly reversibly bound with Thyroxine binding globulins (TBG), and to a lesser extent with albumin and Thyroid binding Pre-Albumin. Thus the conditions in which TBG and protein levels alter such as chronic liver disorders, pregnancy, excess of estrogens, androgens, anabolic steroids and glucocorticoids may cause misleading total T3, total T4 and TSH interpretations.
3. TSH levels may be normal in central hypothyroidism, recent rapid correction of hypothyroidism or hyperthyroidism, pregnancy, phenytoin therapy etc.
4. TSH values of <0.03 uIU/mL must be clinically correlated to evaluate the presence of a rare TSH variant in certain individuals which is undetectable by conventional methods.
5. Presence of Autoimmune disorders may lead to spurious results of thyroid hormones.
6. Various drugs influence the levels of thyroid hormones such as L-Dopa, Lithium, Glucocorticoids, Phenytoin etc.
7. Serum TSH is evaluated in Neonates to diagnose Congenital Hypothyroidism.
8. Within hours of birth, plasma TSH, T4, and T3 concentrations rise rapidly. By 2 to 3 days, TSH concentrations fall. T4 falls to adult concentrations by 1 to 2 months of age.



Dr. Preeti
MBBS, MD Biochemistry
Consultant Biochemist
DMC - 97534, Healthians Labs



SIN No:E6610590

Patient Name	: Miss SNEHA VERMA	Barcode	: E6610590	
Age/Gender	: 21Y 0M 11D /Female	Sample Collected On	: 04/Apr/2026 09:49AM	
Order Id	: 17618795239	Sample Received On	: 04/Apr/2026 03:44PM	
Referred By	: Self	Report Generated On	: 04/Apr/2026 06:59PM	
Customer Since	: 04/Apr/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF IMMUNOLOGY

Test Name	Value	Unit	Bio. Ref Interval
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Vitamin D, 25-Hydroxy

VITAMIN D (25 - OH VITAMIN D)	7.13	ng/ml	30-100
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Method: CLIA

Machine: Beckman Coulter UniCel DxI800

VITAMIN D STATUS	VITAMIN D 25 HYDROXY (ng/mL), Adult	VITAMIN D 25 HYDROXY (ng/mL), Pediatric
DEFICIENCY	<20	<15
INSUFFICIENCY	20 - 30	15 - 20
SUFFICIENCY	30 - 100	20 - 100

Vitamin D is a steroid hormone known for its important role in regulating body levels of calcium and phosphorus and in the mineralization of bone

Uses:

- Diagnosis of Vitamin D deficiency
- Differential diagnosis of causes of rickets and Osteomalacia
- Monitoring Vitamin D replacement therapy
- Diagnosis of hypervitaminosis D

Increased in:

- Vitamin D intoxication
- Excessive exposure to sunlight

Decreased in:

- Malabsorption
- Steatorrhoea
- Dietary osteomalacia
- Thyrotoxicosis
- Coeliac disease
- Inflammatory bowel disease
- Rickets
- Pancreatic insufficiency

*** End Of Report ***



Dr. Preeti
MBBS, MD Biochemistry
Consultant Biochemist
DMC - 97534, Healthians Labs



Dr. Rakesh Kumar
PHD, Biochemistry
Consultant Biochemist
Healthians Labs



SIN No: E6610590

Healthians Labs (A Unit of Expedient Healthcare Marketing Pvt. Ltd.)

Plot 1 & 2, Udyog Vihar, Phase-4, Gurgaon, Haryana, Pincode-122016 (CAP Number: 9019582)

Terms & Conditions:

- 1) Machine Data is available for last 7 days only. In case of manual testing & outsourced testing, machine data will not be available.
- 2) CBC parameters may vary when it is manually reviewed by the Pathologists.
- 3) **For Thyroid tests** - Circulating TSH shows a normal circadian rhythm with a peak between 11pm-5am and a nadir between 5pm-8pm. TSH values are also lowered after food when compared to fasting in a statistically significant manner. This variation is of the order of $\pm 50\%$, hence time of day and fasting status have influence on the reported TSH level.
- 4) **For Lipid profile** - Lipid and Lipoprotein concentrations vary during the normal course of daily activity. Also, certain drugs, diet and alcohol can have lasting effects on Triglyceride levels. To obtain best results for Lipid testing, a strict fasting of 10-12 hours with a light meal on the previous night is recommended.
- 5) Test results released pertain to the specimen submitted.
- 6) Test results are dependent on the quality of the sample received by the Lab.
- 7) The tests are carried out in the lab with the presumption that the specimen belongs to the patient named or identified in the bill/test request form/booking ID.
- 8) The reported results are for information and are subject to confirmation and interpretation by the referring doctor to co-relate clinically.
- 9) Test results may show interlaboratory variations.
- 10) Liability of Healthians for deficiency of services or other errors and omissions shall be limited to the fee paid by the patient for the relevant laboratory services.
- 11) This report is not subject to use for any medico-legal purposes.
- 12) Few of the tests might be outsourced to partner labs as and when required.
- 13) This report is not intended to replace but to lead by providing comprehensive information. It is recommended that you consult your doctor/physician for interpretation of results.
- 14) All reports might not be applicable for individuals less than 18, pregnant women or individuals suffering from diseases for which health test has not been performed or symptoms not diagnosed.
- 15) This report is based on preventive health test screening and is meant for a healthy lifestyle. It does not provide any recommendation for life threatening situations.
- 16) It is strongly recommended to take required precautions for allergic reactions or sensitivities.
- 17) Authorised partner labs as mentioned for certain tests are as below:
HL/PL/001- Metropolis Healthcare Ltd
HL/PL/002- Thyrocare technologies Limited
HL/PL/003- Lifecell International Pvt. Ltd. - Laboratory Services
HL/PL/004- Modern Diagnostic & Research Centre

About Healthians Labs

How we control Report Accuracy at Healthians



Quality Control

We follow Quality control to ensure both **precision & accuracy** of patient results.



Machine Data

We save patient's result values **directly from machines** ensuring no manipulations & no fake values.



QR Code

QR Code based authenticity check on all its reports



Calibration

We make use of calibrators to evaluate the **precision & accuracy** of measurement equipment.



Equipment

Our Labs are equipped with state-of-the-art instruments with **cutting edge technology** to provide faster & reliable results.



EQA

Our Labs participate in EQA & show proven accuracy by checking **laboratory performance** through external agency or facility.

ADVISORY

Health Advisory

Miss SNEHA VERMA

Booking ID : 17618795239 | Sample Collection Date : 04/Apr/2026

22.04 Body Mass Index

5'4" Height (ft/in)

60 Weight (kgs.)



Physical Activity

No Data



Smoke

No Data



Food Preference

No Data



Alcohol

No Data



Medication

No Data



Family History

No Data



Blood Pressure

No Data



Pulse Rate

No Data



Waist (In Cm)

0cm



Hip Circumference (In Cm)

No Data



SPO2 Levels

No Data



Sugar Levels

No Data

Additional Remarks :

NA

SUGGESTED NUTRITION

SUGGESTED NUTRITION

Do's

- Have a balanced diet that includes whole grains, pulses, dairy, fruits, vegetables, nuts and healthy fats
- Include fruits like apples, berries and melons in your diet
- Include calcium rich foods like milk, yoghurt, cheese and green, leafy vegetables
- Include Brazil nuts, sesame seeds, sunflower seeds

Dont's

- Avoid flavoured and seasoned foods
- Decrease intake of colas and sugary drinks
- Avoid saturated fats, trans fats, oily and greasy foods like cakes, creamy or fried foods
- Limit sugar intake
- Reduce caffeine intake

SUGGESTED LIFESTYLE

SUGGESTED LIFESTYLE

Do's

- Have breakfast early in morning and a light high fiber snack for dinner
- Plan a healthy routine and have food at the same time everyday
- Have regular exposure to sunlight

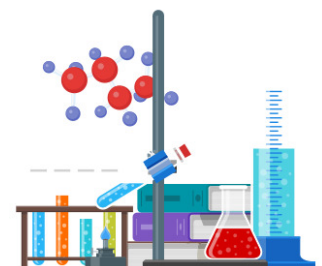
Dont's

- Avoid overexertion without having food or drink
- Avoid strenuous exercises
- Avoid smoking and alcohol
- Don't ignore your body signals and don't skip your regular health check-ups

SUGGESTED FUTURE TESTS

SUGGESTED FUTURE TESTS

- Complete Hemogram - **Every 2 Month**
- Peripheral Smear Examination By Pathologist - **Every 2 Month**
- Vitamin D Total-25 Hydroxy - **Every 2 Month**
- Calcium Total, Serum - **Every 2 Month**



HEALTH ADVISORY

Suggestions for Health & Well-being

Miss SNEHA VERMA

Booking ID : 17618795239 | Sample Collection Date : 04/Apr/2026

PHYSICAL ACTIVITY

PHYSICAL ACTIVITY

Physical activities can vary from Regular walks (Brisk or normal), Jogging , Sports, Stretching, Yoga to light weight lifting etc. It is recommended to partake in physical activity at least 30 minutes a day for 3-4 days a week.

If regular workout is difficult, then we can adapt changes such as using stairs instead of lift/escalators and doing household work!



BALANCED DIET

A balanced diet is the key to healthy lifestyle. Include Whole grains, vegetables, whole fruits, nuts, seeds, beans, plant oils in your diet.

It is recommended to always have a high protein breakfast and a light dinner. Avoid items such as processed foods, potatoes and high calorie/sugar products. Don't forget to drink water regularly!

BALANCED DIET

STRESS MANAGEMENT

STRESS MANAGEMENT

Managing stress is an essential part of well-being. Some day to day changes can help such as having sufficient sleep (6-8 hours), indulging yourself in meditation, positive attitude towards lifestyle, using humor, traveling, talking to people whom you feel comfortable with and making time for hobbies by doing what you love to do.



BMI

BMI recommended range is 18.5 to 24.9. Your BMI is 29.35, which is on a higher side.

BMI INFORMATION NOT AVAILABLE
Please fill your Health Karma to know your BMI results
BMI for your body helps prevent many untimely diseases and goes a long way.

BMI CHART

UNDERWEIGHT	NORMAL	OVERWEIGHT	OBSE
Less than 18.5	Between 18.5 - 24.9	Between 25.0 - 29.9	More than 30
*****	*****	*****	*****

BMI

Supplement Suggestions

Miss SNEHA VERMA

Booking ID : 17618795239 | Sample Collection Date : 04/Apr/2026

Your test report has indicated that you have certain deficiencies in your body which may hamper your health & wellbeing in the longer run.

In order to fulfill the gaps in nutrition and promote a healthier body we suggest you the following supplements mentioned below:

Deficiency/Out of Range Parameter(s)	Suggested Supplement
Vitamin D Total-25 Hydroxy	VITAMIN D3

To order, call 1800-572-000-4

Suggestions for Improving Deficiencies



VITAMIN D3

Improve bone health with enhanced calcium absorption, the natural way

Make your muscles and bones stronger with VITAMIN D3. Sourced from natural substances, it helps in regulating the absorption of calcium and phosphorus, which help keep your bones strong and enhancing the normal immune system functioning. Vitamin D3 is an essential nutrient that's critical for normal growth and development of bones and teeth, as well as improved resistance against certain diseases.

Remember, a lack of vitamin D3 can cause dangerous health situations.

- Rickets (in children) | • Brittle Bones | • Osteoporosis | • Weakened Bones (in adults)

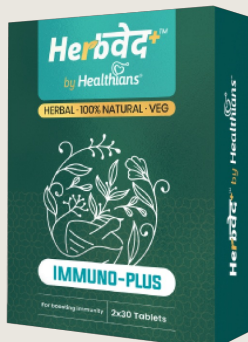
Strengthens Bones & Muscles

Protects Against Pneumonia & Acute Respiratory Infections

Helps in Reducing Depression

Boosts Heart Health

Aids in Kidney Disease Treatment



IMMUNO-PLUS

Give your immunity a boost the all-natural way.

IMMUNO-PLUS is the perfect all-natural herbal supplement to boost your immune system and strengthens your body's defenses against diseases and infections. IMMUNO-PLUS provides your immune system the necessary reinforcement to keep you safe and healthy.

A weakened immune system opens you to a host of illnesses, such as:

- Recurring Infections | • Heightened Risk of Cancer | • Autoimmune Disorders | • Slow Growth Rate | • Serious Damage to the Heart, Lungs, Digestive Tract & the Nervous System

Infused with the ages-proven goodness of all-natural ingredients, IMMUNO-PLUS is the perfect supplement to strengthen your immune system without having to worry about side effects. Sourced from nature's own pharmacy of herbs, the ingredients in IMMUNO-PLUS present the following benefits:

Amla

Boosts immunity & Stores antioxidants

Jetwatika

Antioxidant properties strengthen the immune system

Aloe Vera

Fights against oxygenated rogue molecules in the blood

Ashwagandha

Reinforces the immune system to increase its fighting ability

Ginger

Anti-inflammatory & antioxidant effects reinforce the immune system

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EQA

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